POLICY

Crisis & safety protocol

What to do, in order, when a client may be at risk.

1. **Stay with the client**
   Do not leave a client at risk alone. On telehealth, keep the call open and confirm their location.

2. **Assess risk**
   Use the risk screen in SimplePractice. Ask directly about thoughts, plan, means and intent.

3. **Call your supervisor**
   Call or message your supervisor right away. If they do not answer, call the director.

4. **Make a safety plan or call for help**
   For lower risk, make a written safety plan together. For imminent risk, call 988 or 911.

5. **Document the same day**
   Write a full crisis note before you leave: what happened, the screen, who you called and the plan.

When this protocol applies

Use it whenever a client may be at risk of harming themselves or someone else, in person or on telehealth.

- Thoughts of suicide
- Threats to others
- Acute intoxication or psychosis

Step 1: Stay with the client

Do not leave a client at risk alone. On telehealth, keep the call open and confirm their location.

- Stay calm and present
- Confirm location
- Remove means if safe to do so

Step 2: Assess risk

Use the risk screen in SimplePractice. Ask directly about thoughts, plan, means and intent.

- Thoughts
- Plan and means
- Intent and timeline

Step 3: Call your supervisor

Call or message your supervisor right away. If they do not answer, call the director.

- Supervisor first
- Then the director
- Then the on-call clinician

Step 4: Make a safety plan or call for help

For lower risk, make a written safety plan together. For imminent risk, call 988 or 911.

- Safety plan
- 988 Suicide and Crisis Lifeline
- 911 for imminent danger

Step 5: Document the same day

Write a full crisis note before you leave: what happened, the screen, who you called and the plan.

- Crisis note
- Screen results
- Follow-up booked

Follow-up

Contact the client within 24 hours. Bring the case to your next supervision.

- Check-in within 24 hours
- Review in supervision
- Update the treatment plan